

Ankle Sprains in Children

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The most commonly sprained or strained joint is the ankle. Approximately 1 million ankle injuries occur each year, and 85 percent of them are sprains.

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First Floor, Suite 1150

Waltham, MA 02451

Mass General Brigham Healthcare Center (Foxborough)

20 Patriot Place

Foxborough, MA 02035

Mass General Brigham Healthcare Center (Danvers)

102-104 Endicott Street

Lower Level 00

Danvers, MA 01923

Newton-Wellesley Hospital

978 Worcester Street (Route 9)

Newton, MA 02482

Explore Ankle Sprains

Overview

Treatment

Exercises

Other Treatments

Overview

The most commonly sprained or strained joint is the ankle. Approximately 1 million ankle injuries occur each year, and 85 percent of them are sprains. Ankle sprains can occur during competitive sports or during routine daily activities (walking).

The three ligaments frequently involved in ankle sprains include the following:

Anterior talofibular ligament

Posterior talofibular ligament

Calcaneofibular ligament

The talus bone and the ends of two of the lower leg bones (tibia and fibula) form the ankle joint (see figure 1). The ligamentous structures of the ankle are divided into three groups: the tibiofibular, medial, and lateral complexes. Most ankle sprains happen when the foot turns inward as a person runs, turns, falls, or lands on the ankle after a jump. This common type of sprain is called an inversion injury. The lateral complex consists of three ligaments: anterior talofibular, calcaneofibular, and posterior talofibular. These ligaments are frequently stretched or torn from an inversion ankle sprain. The most frequently injured ligaments are the anterior talofibular followed by the calcaneofibular ligament.

The length of recovery for an ankle sprain can be as long as a fracture (6-10 weeks), and it is important for a patient athlete to wait for healing and rehabilitation to be complete prior to returning to his/her sport. An ankle rehab program is usually done with a physical therapist who will guide the patient back into sports and activities. Completing a formal ankle rehabilitation program will often help avoid re-injuring the ankle.

It is important to note that because children have open growth plates (physis), it is often difficult to differentiate between an ankle sprain and a growth plate fracture. The growth plate appears as a black line on an x-ray which makes it difficult to see a fracture line (which is also a dark line). If there is tenderness directly over the bone where the growth plate is located, then the injury will be treated as a fracture which usually involves a short leg cast or air cast for 4-6 weeks.

Treatment Specifics

Ice: Place a plastic bag with ice on the ankle for 15-20 minutes, 3-5 times/day for the first 24-72 hours. Leave ice off at least 1 1/2 hours between applications.

Compression: Wrap an elastic bandage from the toes up to midcalf, using even pressure. Wear until swelling decreases. Loosen the wrap if toes start to turn blue or feel cold.

Exercises to Restore Normal Ankle Function

Range of Motion Exercises: Help you regain normal ankle motion

Technique: Sit with your knee straight and hold the foot position as long as possible. Do as frequently as possible for the first 3-10 days.

Ankle Alphabet: Make the letters of the alphabet using only the ankle. Repeat 3-4 times and at least 2-3 times/day.

Pullback: Flex your foot back toward your body (can use a towel to assist).

Flexibility (Stretching) Exercises: Loosen tight leg muscles. Tightness makes it hard to use stairs, walk, run and jump.

Technique: Hold each exercise 15 seconds at a gentle stretch. **DO NOT BOUNCE!**

Frequency: 15-20 reps/exercise, 5-7 days/week.

Strengthening Exercises: Thera-Band exercises-resisted inversion, eversion, dorsiflexion, plantarflexion.

Other Treatments

Depending on the severity of the injury, other treatments may include:

Activity Restrictions: In order to avoid re-injury to ankle, a patient may need to complete functional testing pain free prior to return to sports (jogging forward/backward, figure eights, one-foot hops, etc)

Splint/Cast: A splint or cast protects the ankle to prevent re-injury while ligaments heal

Crutches/Wheelchair: Crutches are needed until patient is able to bear weight without pain

Physical Therapy: (to stretch and strengthen the injured muscles, ligaments, and tendons). It is very important to complete ankle rehabilitation program prior to return to sports to avoid re-injuring the ankle

Surgery: Surgery may be necessary if there is recurrent instability, or if a muscle, tendon, or ligament is badly torn

Meet our Children's Orthopaedic Team

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Have questions about pediatric ankle sprains? Get in touch.

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